جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : اسماء عبدالوهاب محمد جوده
المحافظة : القاهره **الحي / المدينة :** المطريه **العنوان :** //
رقم البطاقة : 26710191701041 **رقم التليفون :** 01116214780 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 19-10-1967 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : ورم بالغدة الدرقية
نتائج الأبحاث : (مرفق)
التشخيص النهائي : ورم بالغدة الدرقية
خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي [6873] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: اسماء عبدالوهاب محمد جوده
National ID: 26710191701041

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of thyroid gland tumor for Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

National ID – Front
[[DOCTYPE.ID_FRONT]]



National ID – Back
[[DOCTYPE:ID_BACK]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : اسماء عبد الوهاب محمد جوده **Analysis ID :** 1101999

Age : 58 Y : 1 M : 1 D **Gender :** Female **Conf. Code :** 0400

Referred By Prof. Dr. : على عزمى

Clinical Diagnosis : Enlarged posterior cervical lymph node

Nature of Specimen : U/S guided tru- cut biopsy and U/S guided aspiration

Receiving Date : 2026-10-06 **Patient ID :** 802600

PATHOLOGY REPORT

Gross :

Two specimens were received:
1- Multiple tan greyish white tissue cores, the longest measured 1 cm, totally submitted.
2- 7 ml of dark blackish fluid.

Microscopic :

Sections examined from the specimen received revealed cores of nodal tissue showing metastatic tumor tissue formed of groups of malignant epithelial cells showing moderate nuclear enlargement, overlapping and intranuclear inclusions arranged in papillary pattern. There are scattered psammoma bodies.
There is mild desmoplasia.
Cytological examination of the films and cell block prepared from the centrifugate of the fluid received revealed eosinophilic proteinaceous material, scattered lymphocytes and foamy macrophages.

Diagnosis :

Posterior cervical lymph node, U/S guided tru- cut biopsy and U/S guided aspiration, Both specimens, METASTATIC PAPILLARY THYROID CARCINOMA.
-IMMUNOPHENOTYPING IN A SEPARATE REPORT.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

This report is electronically signed , any abrasion or modification would render it void .

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥

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Member , United States Canadian Academy of Pathology

Patient Name : اسماء عبد الوهاب محمد جوده **Analysis ID :** 1102000

Age : 58 Y : 1 M : 1 D **Gender :** Female **Conf. Code :** 9040

Referred By Prof. Dr. : على عزمى

Clinical Diagnosis : Enlarged posterior cervical lymph node

Nature of Specimen : Block (Labelled as 1101999, 1), U/S guided tru- cut biopsy)

Receiving Date : 2026-10-06 **Patient ID :** 802600

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain .
The sections were treated against:
- TTF1 \ PAX8

Results :

Tumor cells are POSITIVE for TTF1.
Tumor cells are POSITIVE for PAX8 .

Conclusion :

Block, FINDINGS ARE COMPATIBLE WITH METASTATIC PAPILLARY THYROID CARCINOMA.

Prof. Dr. Shady Elia Anis *Prof. Dr. Elia Anis Ishak*

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عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥

Name	Mrs. Asmaa Abd El-Wahhab Mohamed Gouda		
Ref. By Prof. Dr.	Ali Mohamed Azmy	Date	24/9/2026
ID	9926017654	Acc. No.	240342222

^{18}F-FDG PET/CT Scan

Indication of the study: Status evaluation study for a case with previous history of ? lymph nodal neoplasm (as per se). No available previous PET/CT studies or pathological data (patient refused to supply any).

Total body PET scan was done 40 minutes after IV administration of 300 MBq of 18F-FDG.

CT scanning was performed simultaneously without IV contrast material injection.

Semi-quantitative analysis would be done for selected ROI.s & the normal threshold would be the normal blood pool liver $SUV_{max.}$ < 2.3

Head & Neck:

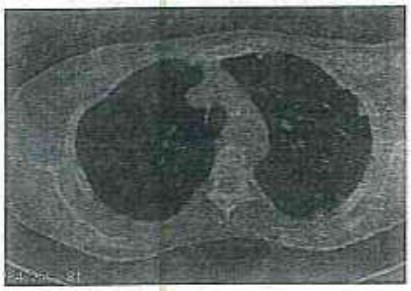
- Normal physiological biodistribution over scanned portion of the brain.
  (N.B; not all brain metastatic lesions could be detected on 18-FDG PET/CT due to physiologic uptake in brain. MRI brain is suggested in cases of clinical suspicion).
- Normal naso-/oro-pharynx, larynx, thyroid & salivary glands.
- *Low activity right upper cervical rather cystic oval lesion about 3.5 X 2 cm of SUVmax. 0.8.*



Torso:

Thorax:

- Low activity bilateral apical pleural thickening and low activity subpleural reticulations, reaching up to 8 mm of SUVmax. 0.5; mostly of granulomatous nature than otherwise; for follow-up.
- Few bilateral sub-pleural non-metabolically active nodules, largest ± 4-6 mm, mostly non-specific, however; for follow up.
- Bilateral tracer starved atelectasis and sub-pleural reticulations.

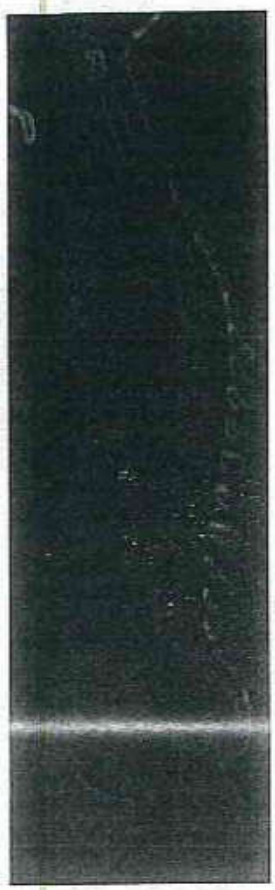


- No FDG hypermetabolic pulmonary nodules or masses seen.
- No enlarged FDG hypermetabolic hilar or mediastinal L.Ns seen.
- No pleural or pericardial effusion seen.

Abdomen & Pelvis:

- Enlarged liver with non-uniform activity yet no FDG hypermetabolic focal lesions, mostly blood pooling. For dynamic MRI study of the liver assessment if warranted.

Name	Mrs. Asmaa Abd El-Wahhab Mohamed Gouda		
Ref. By Prof. Dr.	Ali Mohamed Azmy	Date	24/9/2026
ID	9926017654	Acc. No.	240342222



- No biliary tree dilatation.
- Average size of the spleen with no focal active lesions.
- Normal appearance of pancreas (no sizable active masses) and adrenals.
- *Low activity left inguinal rather cystic lesion about 2 cm of SUVmax. 0.5.*
- Congested mesentery with strandy reticulations.
- Normal physiological biodistribution over the bowel (non-uniform bowel uptake with no sizable masses) & urinary tracts (ptosed / low lying right kidney).



Surveyed skeleton:

- Equivocal marrow uptake of SUVmax. 2.3 yet no tracer active lytic or sclerotic osseous lesions seen, likely hematopoietic, for CBC correlation and follow up.
- Scattered sclerotic non-metabolically active foci of the spine and pelvic bones (mostly degenerative changes).

Opinion:

- Low activity cervical and inguinal lesions (? Necrotic lymph nodes), for unavailable data correlation.
- Rest of the surveyed body is free from any other FDG hypermetabolic neoplastic masses.
- The left inguinal region would be considered the most amenable for biopsy.

Thanks for your referral;

Dr. Islam Aboulwafa, MD — **Consultant Radiologist**

H.S 26/09/26 6:56:09 PM

Name: Mrs. Asmaa Abd El-Wahhab Mohamed Gouda

Date: 6/10/2026 ***ID:9926017654*** ***Acc.:240344425***

Referring Prof. Dr. Ali Mohamed Azmy

Right posterior cervical lymph node biopsy

- Us examination revealed enlarged posterior cervical lymph node with septations and cystic degeneration.
- Us guided Aspiration of fluid inside revealed 10 cc of bloody fluid for histopathological correlation.
- Us guided tru cut biopsy of the soft tissue inside the lymph node done with 4 cores obtained for histopathological correlation.

Best regards;

Dr. Mahmoud Ibrahim, MSc

Spelling checked by: Dr. M.I.
06/10/26 2:02:17 PM

Name : ASMAA ABDEL WAHAB.
Date : 10-9-2026.

Thyroid U.S.

- **Right lower posterior cervical lymph node measuring 5.5x3.4cm with increased cortical thickness and cystic degenerations.**
- **The right thyroid lobe** average in size measures 13x14x28 mm with homogeneous echo pattern, no nodules , no evidence of calcifications or cystic degenerations.
- **The left thyroid lobe** average in size measures 12x15x31 mm with homogeneous echo pattern, no nodules , no evidence of calcifications or cystic degenerations. , no abnormal vascularity on color Doppler study.
- **The isthmus** average in size measuring 2mm.
- Average vasculature.
- No retro-sternal extension.

Conclusion.

Right lower posterior cervical ?suspicious lymphadenopathy for further ultrasound guided tru cut needle biopsy

Much Obliged, Yours
Dr/ENAS HUSSIEN , MD.

Mrs.: ASMAA ABDEL WAHAB
Date: 10/09/26

Digitized Mammography and Complementary Ultrasonography for both breasts revealed:

Findings:

- **Condensed mammary parenchyma; ACR c with prominent fibro glandular component causing mottling appearance of fibroadenosis.**
- **bilateral breast upper outer and lower inner quadrant oval shaped isodense mass lesion with no surrounding soft tissue edema. .**
- No pathological micro calcifications visualized.
- Normal both skin thickness and contour.
- Bilateral non-specific axillary nodes are seen.

Bilateral Breast U / S Revealed

- Normal diffusely prominent fibro-glandular breast parenchyma.
- **Multiple bilateral benign looking breast masses at right breast 12 o'clock measuring 8x5mm away from the skin by 5mm and at 7 o'clock measuring 3x7mm away from the skin by 8mm , at left breast oval shaped mass lesion located at 7 o'clock measuring 4x7mm away from the skin by 1cm & at retroareolar tissue measuring 4x6mm with no surrounding subcutaneous and soft tissue edema. with some posterior shadowing**
- Normal thickness of the skin of both breast .
- Normal both nipples ,areola complex .
- Bilateral non specific axillary lymph-adenopathy the largest one on the right axilla measures about 12 x 8 mm and on the left axilla measures about 11 x 9 mm all with preserved echogenic fatty hillum.

Conclusions

- Bilateral breast benign looking mass lesion correlated to BIRADS 3with for further follow up study
- Bilateral fibroadenosis .

Much obliged
Dr. ENAS HUSSIEN ,MD

Breast Imaging Reporting and Database System (BI-RADS®)		
Follow-up Recommendations	**Assessment**	**Category**
a. Assessment is Incomplete		
Additional imaging and/or prior images are needed before a final assessment can be assigned	Need Additional Imaging Evaluation and/or Prior Mammograms for Comparison	0
b. Assessment is Complete – Final Categories		
Routine annual screening mammography (for women over age 40)	Negative	1
Routine annual screening mammography (for women over age 40)	Benign Finding(s)	2
Initial short-term follow up (usually 6-month) examination	Probably Benign Finding – Initial Short-Interval Follow-Up Suggested	3
Usually requires biopsy	Suspicious Abnormality – Biopsy Should Be Considered Optional subdivisions:* 4A: Finding needing intervention with a low suspicion for malignancy 4B: Lesions with an intermediate suspicion of malignancy 4C: Findings of moderate concern, but not classic for malignancy	4
Requires biopsy or surgical treatment	Highly Suggestive of Malignancy – Appropriate Action Should Be Taken	5
Category reserved for lesions identified on imaging study with biopsy proof of malignancy prior to definitive therapy	Known Biopsy-Proven Malignancy – Appropriate Action Should Be Taken	6

العنوان: ٢٠ ش العزيز بالله – الزيتون – القاهرة – محطة مترو "حدائق الزيتون"
ت: ٠٢ ٢٤٥٤٤٠١١ / ٠٢ ٢٤٥١٨٠٩٨ داخلي ٣٠٢ موبايل: ٠١١٤٨٥٥٨٢٢٢

Patient Name	السيدة/ اسماء عبدالوهاب محمد جوده	Patient ID.	1026091610125
Age / Sex	57 Years / Female	Request Date	16-Sep-2026 03:51 PM
Referred By	أ.د/ عمر عثمان احمد	Printed In	17-Sep-2026 08:31 PM

HORMONE

			Reference Range :
Free T4 (Free Thyroxine)	:	1.28	0.85 - 1.85 ng/dL
TSH (Thyroid Stimulating Hormone)	:	3.15	0.35 - 5.10 uIU/mL

Doctor's signature:

Thanks

Patient Name	السيدة/ اسماء عبدالوهاب محمد جوده	Patient ID.	1026091610125
Age / Sex	57 Years / Female	Request Date	16-Sep-2026 03:51 PM
Referred By	أ.د/ عمر عثمان احمد	Printed In	17-Sep-2026 07:49 PM

IMMUNOLOGY

		Ref. Range :
⚠ HBsAg (Hepatitis B Surface Antigen) :	Negative	Negative
⚠ HCV (Hepatitis C Virus Ab 3rd Generation)	Equivocal 0.96	Negative <0.9 Equivocal 0.9 - 1.1 Positive >1.1

Human Immunodeficiency Virus:

⚠ HIV Antibody :	Negative	Negative

Comment : HCV IgG have to be repeated after 6.- 8 weeks. HCV RNA by PCR is recommended.

Doctor's signature:

Thanks

Patient Name	السيدة/ اسماء عبدالوهاب محمد جوده	Patient ID.	1026091610125
Age / Sex	57 Years / Female	Request Date	16-Sep-2026 03:51 PM
Referred By	أ.د/ عمر عثمان احمد	Printed In	17-Sep-2026 07:51 PM

HEMATOLOGY

Blood Picture

				Reference Range :
Hgb (Hemoglobin)	: 12.9	g/dL		12.5 - 16.0 g/dL
RBCs (Red Blood Cells)	: 4.39			4.2 - 5.4 x 10^3 Cells/µL
Hct (Hematocrit)	: 34.1		L	37 - 47 %
MCV	: 77.7		L	78 - 99 fL/cell
MCH	: 29.4			27 - 31 pg/cell
MCHC	: 37.8		H	32 - 36 g/dL
RDW	: 14.5		H	11.6 - 14.0 %
Plt (Platelet)	: 295			140 - 440 x 10^3/mm^3
WBCs (Leukocytes)	: 8.5			4.0 - 11.0 x 10^3/mm^3

Test		Relative count %		Absolute count K/uL	
Neutrophil	: 69		35 - 80	5.86	2.0 - 6.5
Segmented	: 64		35 - 80	5.44	1.6 - 7.1
Band	: 5		0 - 11.0	0.42	0 - 1.2
Lymphocytes	: 20		18 - 44	1.7	1.5 - 3.5
Monocytes	: 8		0 - 10	0.68	0.3 - 1.0
Eosinophils	: 3		0 - 3	0.26	0 - 0.5
Basophils	: 0		0 - 1	0	0 - 0.1

Comment : The count is within normal limit for age & sex, PLTs are Normal.

Doctor's signature:

Thanks

Patient Name	السيدة/ اسماء عبدالوهاب محمد جوده	Patient ID.	1026091610125
Age / Sex	57 Years / Female	Request Date	16-Sep-2026 03:51 PM
Referred By	أ.د/ عمر عثمان احمد	Printed In	17-Sep-2026 07:52 PM

CHEMISTRY

			Reference Range :
SGOT (Aspartate Aminotransferase)	: 15	U/L	Normal Up to 31
SGPT (Alanine Aminotransferase)	: 10	U/L	Normal Up to 31
Serum Creatinine	: 0.67	mg/dL	Normal Up to 1.30
Blood Urea	: 26	mg/dL	Normal 15 - 45
HbA1C (Glycosylated Hemoglobin)	: 6.0	%	Non Diabetic : 4.5 - 5.6 % Prediabetic : 5.7 - 6.5 Diabetic > 6.5 Controlled Diabetic < 7 Uncontrolled Diabetic > 8
eAG	: 126		

Doctor's signature:

Thanks

Patient Name	السيدة/ اسماء عبدالوهاب محمد جوده	Patient ID.	1026091610125
Age / Sex	57 Years / Female	Request Date	16-Sep-2026 03:51 PM
Referred By	أ.د/ عمر عثمان احمد	Printed In	17-Sep-2026 08:29 PM

IMMUNOLOGY

Tumors Markers:

			Reference Range :
CEA (Carcinoembryonic Antigen)	:	1.2	Non smoker Up to 5.0 ng/mL smoker Up to 5.5
CA 19.9	:	22.8	Up to 27.5 U/mL
CA 125	:	14.5	Up to 37.2 U/mL

Doctor's signature:

Thanks

Patient Name	السيدة/ اسماء عبدالوهاب محمد جوده	Patient ID.	1026091610125
Age / Sex	57 Years / Female	Request Date	16-Sep-2026 03:51 PM
Referred By	أ.د/ عمر عثمان احمد	Printed In	17-Sep-2026 09:07 PM

CHEMISTRY

Vit D (25 Dihydroxycolycaliciferol):

⚠ 25 Hydroxy Vitamin D	:	7.0	

Reference Range :

Deficiency < 12 ng/mL
Insufficiency 12 - 29
Sufficiency >=30
Toxicity >=150

Doctor's signature:

Thanks

Patient Name	السيدة/ اسماء عبدالوهاب محمد جوده	Patient ID.	1 026091 610125
Age / Sex	57 Years / Female	Request Date	16-Sep-2026 03:51 PM
Referred By	أ.د/ عمر عثمان احمد	Printed In	17-Sep-2026 08:32 PM

HEMATOLOGY

				Ref. Range :
PT (Prothrombin Time)				
Patient's time	:	13.0	sec	Female 11 - 14 sec
Control's time	:	13	sec	
Concentration	:	100	%	Female 70 - 100 %
I.N.R.	:	1		Female 1.0 - 1.3
Ratio	:	1		

Doctor's signature:

Thanks

Patient Name	السيدة/ اسماء عبدالوهاب محمد جوده	Patient ID.	1026091610125
Age / Sex	57 Years / Female	Request Date	16-Sep-2026 03:51 PM
Referred By	أ.د/ عمر عثمان احمد	Printed In	17-Sep-2026 08:53 PM

HEMATOLOGY

Westergren Method For: | **Reference Range:**

✧ ESR (Erythrocyte Sedimentation Rate)

⏃ First Hour	:	27	mms	Normal: Up to 12 mms
⏃ Second Hour	:	60	mms	

Doctor's signature:

Thanks